

Processed At :
Sohrabh Hall, 112, A Wing, 1st
Floor, Sangamwadi, Pune 411001

Your ABHA Integrated Personal Digital Health Partner

REPORT

Patient Name : SRISHTI RISHI(28Y/F)
Referred By : SELF
Address : , C-308, C- Wing, Green Life Society, Laxmi chowk , behind tata
motors Hinjewadi phase 1, Pu , Pune , Pune , MAHARASHTRA , India ,
411057

Tests Done : COMPLETE HEALTH CHECK WITH
VITAMINS

Report Availability Summary

Note: Please refer to the table below for status of your tests.

 **16** Ready  **0** Ready with Cancellation  **0** Processing  **0** Cancelled in Lab

TEST DETAILS

REPORT STATUS

COMPLETE HEALTH CHECK WITH VITAMINS	Ready 
CARCINO EMBRYONIC ANTIGEN (CEA)	Ready 
CHLORIDE	Ready 
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready 
Lipoprotein (a) [Lp(a)]	Ready 
SODIUM	Ready 
COMPLETE URINE ANALYSIS	Ready 
HBA PROFILE	Ready 
HEMOGRAM - 6 PART (DIFF)	Ready 
LIVER FUNCTION TESTS	Ready 
ELEMENTS 22 (TOXIC AND NUTRIENTS)	Ready 
IRON DEFICIENCY PROFILE	Ready 
KIDPRO	Ready 
LIPID PROFILE	Ready 
T3-T4-USTSH	Ready 
VITAMIN D TOTAL AND B12 COMBO	Ready 
APOLIPROTEIN RATIO	Ready 

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Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
LYMPHOCYTE	45.8	%	20-40
LYMPHOCYTES - ABSOLUTE COUNT	3.7	X 10 ³ / µL	1.0-3.0
MEAN CORP.HEMO.CONC(MCHC)	31	g/dL	31.5-34.5
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	26.9	pq	27.0-32.0
MONOCYTES - ABSOLUTE COUNT	0.18	X 10 ³ / µL	0.2 - 1.0
TOTAL RBC	5.06	X 10 ⁶ /µL	3.8-4.8
IRON DEFICIENCY			
% TRANSFERRIN SATURATION	6.41	%	13 - 45
IRON	30	µg/dL	50 - 170
UNSAT.IRON-BINDING CAPACITY(UIBC)	438	µg/dL	162 - 368
LIPID			
LDL / HDL RATIO	1.3	Ratio	1.5-3.5
TC/ HDL CHOLESTEROL RATIO	2.5	Ratio	3 - 5
RENAL			
BLOOD UREA NITROGEN (BUN)	5.98	mg/dL	7.94 - 20.07
UREA (CALCULATED)	12.8	mg/dL	Adult : 17-43
VITAMINS			
25-OH VITAMIN D (TOTAL)	28.43	ng/mL	30-100

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Sample Collected on (SCT) : 19 Apr 2026 07:05
Sample Received on (SRT) : 19 Apr 2026 10:09
Report Released on (RRT) : 19 Apr 2026 12:17
Sample Type | Barcode : URINE | FH825814

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
<u>Physical Examination</u>				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.010	-	1.003-1.030
PH	pH indicator	6	-	5-8
<u>Chemical Examination</u>				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
<u>Microscopic Examination</u>				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-2
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	ABSENT	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase)

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr. Zeeshan Hashmi

Dr Zeeshan Hashmi
MD(Path)



Scan QR to verify(valid for
30 days from release time)

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Sample Collected on (SCT) : 19 Apr 2026 07:05
Sample Received on (SRT) : 19 Apr 2026 10:36
Report Released on (RRT) : 19 Apr 2026 14:01
Sample Type | Barcode : SERUM | FH592512

TEST NAME	TECHNOLOGY	VALUE	UNITS
CARCINO EMBRYONIC ANTIGEN (CEA) Bio. Ref. Interval. :-	C.L.I.A	0.81	ng/mL

Non-Smokers : < 2.50 ng/mL
Smokers : < 5.00 ng/mL

Clinical Significance :

CEA is often used to monitor patients with cancers of the gastrointestinal tract (GI). Increased CEA levels can indicate some Non-Cancer related conditions, Such as some forms of inflammation, Cirrhosis, and Peptic Ulcer. Also, Smokers tend to have Higher CEA levels than Non-Smokers. When cancer spreads to other organs, CEA levels rise and may be present in other types of bodily fluids besides blood.

For Diagnostic Purpose, Results should always be assessed in Conjunction with the patients medical history, clinical examination and other findings.

Specifications:

Precision: Intra Assay (%CV): 3.6 %, Inter Assay (%CV): 4.1 %; Sensitivity: 0.5 ng/ml

Kit Validation References:

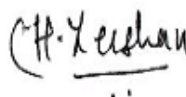
Statland Be, Winkel P. Neoplasia. In: Kaplan LA, Resc AJ, Editors. Clinical Chemistry, Theory, Analysis and Correlation. 2nd Ed. St. Louis: Cv Mosby, 1989.p 734-5.

Please correlate with clinical conditions.

Method:- FULLY AUTOMATED TWO STEP SANDWICH IMMUNOASSAY

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-5319



Dr Zeeshan Hashmi
MD(Path)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	C.L.I.A	28.43	ng/mL

Bio. Ref. Interval. :

DEFICIENCY : <20 ng/ml || INSUFFICIENCY : 20-<30 ng/ml
SUFFICIENCY : 30-100 ng/ml || TOXICITY : >100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml.

Kit Validation Reference: Holick MF. Vitamin D Deficiency. N Engl J Med. 2007;357:266-81.

Method : Fully Automated Chemi Luminescent Immuno Assay

VITAMIN B-12	C.L.I.A	606	pg/mL
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Bio. Ref. Interval. :

Normal : 211 - 911 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:

Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569-73.

Method : COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

Please correlate with clinical conditions.

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

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TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1) Bio. Ref. Interval. : Male : 86 - 152 Female : 94 - 162 Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER	IMMUNOTURBIDIMETRY	122	mg/dL
APOLIPOPROTEIN - B (APO-B) Bio. Ref. Interval. : Male : 56 - 145 Female : 53 - 138 Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER	IMMUNOTURBIDIMETRY	63	mg/dL
APO B / APO A1 RATIO (APO B/A1) Bio. Ref. Interval. : Male : 0.40 - 1.26 Female : 0.38 - 1.14 Clinical Significance : • Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol. • Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD. • The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc. • An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk. • All results have to be interpreted in Conjunction with clinical history and other findings. Method : Derived from serum Apo A1 and Apo B values	CALCULATED	0.5	Ratio

Please correlate with clinical conditions.

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

(H. Zeeshan)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
LIPOPROTEIN (A) [LP(A)] Bio. Ref. Interval. :-	IMMUNOTURBIDIMETRY	27.1	mg/dL

Adults : < 30.0 mg/dl

Clinical Significance:

Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:

Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:

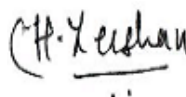
Tietz NW, Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.

Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP) Bio. Ref. Interval. :-	IMMUNOTURBIDIMETRY	1.6	mg/L

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:

High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

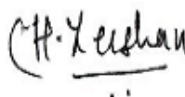
Kit Validation Reference:

- 1.Clinical management of laboratory data in medical practice 2003-3004, 207(2003).
- 2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.

Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS



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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 Method : Ferrozine method without deproteinization	PHOTOMETRY	30	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl Method : Spectrophotometric Assay	PHOTOMETRY	468	µg/dL
% TRANSFERRIN SATURATION Bio. Ref. Interval. : 13 - 45 Method : Derived from IRON and TIBC values	CALCULATED	6.41	%
UNSAT.IRON-BINDING CAPACITY(UIBC) Bio. Ref. Interval. : 162 - 368 Method : SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	438	µg/dL
Please correlate with clinical conditions.			

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

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(H. Zeeshan)

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	139	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	56	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	72	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	112	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	2.5	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	2.01	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	1.3	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.78	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	83.2	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	22.46	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDH - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

*REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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CH. Zeeshan

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	76.7	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.68	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.14	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.54	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	17.6	U/L	< 38
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	27.1	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	30	U/L	< 34
SGOT / SGPT RATIO	CALCULATED	0.9	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	6.77	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	3.9	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	2.87	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.36	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SODIUM Bio. Ref. Interval. : ADULTS: 136-145 MMOL/L Method : ION SELECTIVE ELECTRODE - INDIRECT	I.S.E - INDIRECT	140.1	mmol/L
CHLORIDE Bio. Ref. Interval. : ADULTS: 98-107 MMOL/L	I.S.E - INDIRECT	104.8	mmol/L

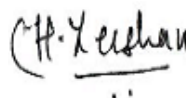
Clinical Significance :

An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).

Method : ION SELECTIVE ELECTRODE - INDIRECT

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	5.98	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.63	mg/dL	0.55-1.02
BUN / SR.CREATININE RATIO	CALCULATED	9.49	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	12.8	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	20.31	Ratio	< 52
URIC ACID	PHOTOMETRY	3.51	mg/dL	3.2 - 6.1
CALCIUM	PHOTOMETRY	8.98	mg/dL	8.8-10.6

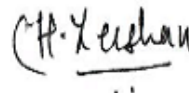
Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
URIC - Uricase / Peroxidase Method
CALC - Arsenazo III Method, End Point.

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd



Dr Zeeshan Hashmi
MD(Path)

Your ABHA Integrated Personal Digital Health Partner

REPORT

Patient Name : SRISHTI RISHI(28Y/F)
Referred By : SELF
Address : , C-308, C- Wing, Green Life Society, Laxmi chowk , behind
tata motors Hinjewadi phase 1, Pu , Pune , Pune ,
MAHARASHTRA , India , 411057

Sample Collected on (SCT) : 19 Apr 2026 07:05
Sample Received on (SRT) : 19 Apr 2026 10:36
Report Released on (RRT) : 19 Apr 2026 14:01
Sample Type | Barcode : SERUM | FH592512

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	107	ng/dL	60-200
TOTAL THYROXINE (T4)	C.L.I.A	8.3	µg/dL	4.5-12
TSH - ULTRASENSITIVE	C.L.I.A	4.173	µIU/mL	0.55-4.78

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
USTSH - Third Generation Ultrasensitive Chemi Luminescent Immuno Assay

Pregnancy reference ranges for TSH/USTSH :

Trimester || T3 (ng/dl) || T4 (µg/dl) || TSH/USTSH (µIU/ml)

1st || 83.9-196.6 || 4.4-11.5 || 0.1-2.5

2nd || 86.1-217.4 || 4.9-12.2 || 0.2-3.0

3rd || 79.9-186 || 5.1-13.2 || 0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-5319

Dr. Zeeshan Hashmi

Dr Zeeshan Hashmi
MD(Path)

Your ABHA Integrated Personal Digital Health Partner

REPORT

Patient Name : SRISHTI RISHI(28Y/F)
Referred By : SELF
Address : , C-308, C- Wing, Green Life Society, Laxmi chowk , behind
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Report Released on (RRT) : 19 Apr 2026 14:01
Sample Type | Barcode : SERUM | FH592512

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR) Bio. Ref. Interval. :-	CALCULATED	124	mL/min/1.73 m2

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

CH. Zeeshan

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-5319

Dr Zeeshan Hashmi
MD(Path)

Your ABHA Integrated Personal Digital Health Partner

REPORT

Patient Name : SRISHTI RISHI(28Y/F)
Referred By : SELF
Address : , C-308, C- Wing, Green Life Society, Laxmi chowk , behind
tata motors Hinjewadi phase 1, Pu , Pune , Pune ,
MAHARASHTRA , India , 411057

Sample Collected on (SCT) : 19 Apr 2026 07:05
Sample Received on (SRT) : 19 Apr 2026 19:44
Report Released on (RRT) : 20 Apr 2026 03:36
Sample Type | Barcode : EDTA Whole Blood | FK166286

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ARSENIC	ICP-MS	0.24	µg/L	< 5
CADMIUM	ICP-MS	0.77	µg/L	< 1.5
MERCURY	ICP-MS	0.11	µg/L	< 5
LEAD	ICP-MS	19.56	µg/L	< 150
CHROMIUM	ICP-MS	1.01	µg/L	< 30
BARIUM	ICP-MS	0.2	µg/L	< 30
COBALT	ICP-MS	0.5	µg/L	0.10 - 1.50
CAESIUM	ICP-MS	1.63	µg/L	< 5
THALLIUM	ICP-MS	0.04	µg/L	< 1
URANIUM	ICP-MS	0.05	µg/L	< 1
STRONTIUM	ICP-MS	14.19	µg/L	8 - 38
ANTIMONY	ICP-MS	4.73	µg/L	0.10 - 18
TIN	ICP-MS	0.29	µg/L	< 2
MOLYBDENUM	ICP-MS	0.78	µg/L	0.70 - 4.0
SILVER	ICP-MS	0.23	µg/L	< 4
VANADIUM	ICP-MS	0.37	µg/L	< 0.8
BERYLLIUM	ICP-MS	0.04	µg/L	<0.10
BISMUTH	ICP-MS	0.17	µg/L	0.10 - 0.80
SELENIUM	ICP-MS	83.26	µg/L	60 - 340
ALUMINIUM	ICP-MS	4.64	µg/L	< 30
NICKEL	ICP-MS	0.97	µg/L	< 15
MANGANESE	ICP-MS	12.16	µg/L	7.10 - 20

Please correlate with clinical conditions.

Method :

ICP - MASS SPECTROMETRY

Note:Reference range has been obtained after considering 95% population as cutoff.

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-2407

Dr Arshiya MD(Path)

Dr Ranjana Roy
MD(Path)

Your ABHA Integrated Personal Digital Health Partner

REPORT

Patient Name : SRISHTI RISHI(28Y/F)
Referred By : SELF
Address : , C-308, C- Wing, Green Life Society, Laxmi chowk , behind
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MAHARASHTRA , India , 411057

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Report Released on (RRT) : 20 Apr 2026 03:36
Sample Type | Barcode : EDTA Whole Blood | FK166286

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.2	%

Bio. Ref. Interval. :

As per ADA Guidelines

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
>=6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	103	mg/dL
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Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-2407

Arshiya Roy

Dr Arshiya MD(Path)

R Roy

Dr Ranjana Roy
MD(Path)

Your ABHA Integrated Personal Digital Health Partner

REPORT

Patient Name : SRISHTI RISHI(28Y/F)
Referred By : SELF
Address : , C-308, C- Wing, Green Life Society, Laxmi chowk , behind
tata motors Hinjewadi phase 1, Pu , Pune , Pune ,
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Sample Collected on (SCT) : 19 Apr 2026 07:05
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Sample Type | Barcode : EDTA Whole Blood | FK166286

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	13.6	g/dL	12.0-15.0
Hematocrit (PCV)	CPH Detection	43.9	%	36.0-46.0
Total RBC	HF & EI	5.06	X 10⁶/μL	3.8-4.8
Mean Corpuscular Volume (MCV)	Calculated	86.8	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	26.9	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	31	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	43.7	fL	39.0-46.0
Red Cell Distribution Width (RDW - CV)	Calculated	13.9	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	238.4	-	*Refer Note below
MENTZER INDEX	Calculated	17.2	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	8.07	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	50.2	%	40-80
Lymphocytes Percentage	Flow Cytometry	45.8	%	20-40
Monocytes Percentage	Flow Cytometry	2.2	%	2-10
Eosinophils Percentage	Flow Cytometry	1.1	%	1-6
Basophils Percentage	Flow Cytometry	0.5	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	4.05	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	3.7	X 10³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.18	X 10³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.04	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.09	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10 ³ / μL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	303	X 10 ³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	10.9	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	12.8	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	32.4	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.33	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

~~ End of report ~~

Tests Done : COMPLETE HEALTH CHECK WITH VITAMINS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-2407

Arshiya Dose

Dr Arshiya MD(Path)

R Roy

Dr Ranjana Roy
MD(Path)



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30 days from release time)